

PACKAGE INSERT

DIATROL TABLET

Each tablet contains:
Loperamide HCl 2 mg

Pharmacology:

In man, loperamide prolongs the transit time of the intestinal contents. It reduces the daily faecal volume, increases the viscosity and bulk density, and diminishes the loss of fluid and electrolytes by both increasing the intestinal absorption and decreasing intestinal secretions. Tolerance to the anti-diarrhoeal effects has not been observed. Clinical studies have indicated that the apparent elimination half life of loperamide in man is 10.8 hours with a range of 9.1 to 14.4 hours. Plasma levels of unchanged drug remain below 2 nanograms per ml after the intake of 2mg of Loperamide Hydrochloride. Plasma levels are highest approximately five hours after administration of the capsule dosage form. Of the total excreted in urine and faeces, most of the administered drug was excreted in faeces.

Indications:

Loperamide HCl is indicated for the control and symptomatic relief of acute, nonspecific diarrhoea and of chronic diarrhoea associated with inflammatory bowel disease. In patients with ileostomy it can be used to reduce the number and volume of stools and to harden their consistency.

Side Effects/Adverse Reactions:

Adverse experiences recorded during clinical studies with Loperamide HCl were generally of a minor and self-limiting nature. They were more commonly observed during the treatment of chronic diarrhoea. The following complaints have been reported : abdominal pain, constipation, drowsiness or dizziness, dry mouth, nausea and vomiting, tiredness, hypersensitivity reactions (including skin rash). Post-marketing Experience: Cardiac Disorders: QT/QTc interval prolongation, Torsades de Pointes, other ventricular arrhythmias, cardiac arrest, syncope, and death (see Warnings and Precautions).

Precautions/Warnings:

Antiperistaltic agents should not be used in acute diarrhoea associated with organisms that penetrate the intestinal mucosa eg. enteroinvasive E. Coli, Salmonella, Shigella. Fluid and electrolytes depletion may occur in patients who have diarrhoea. The use of loperamide does not preclude the administration of appropriate fluid and electrolyte therapy. Appropriate fluid and electrolyte therapy should be given to protect against dehydration in all cases of diarrhoea. Oral rehydration therapy - which is the use of appropriate fluids including oral rehydration salts - remains the most effective treatment for dehydration due to diarrhoea. The intake of as much of these fluids as possible is therefore imperative. Drug - induced inhibition of peristalsis may result in fluid retention in the intestine which may aggravate and mask dehydration and depletion of electrolytes. If severe dehydration or electrolyte imbalance is present loperamide should be withheld until appropriate corrective therapy has been initiated. In acute diarrhoea, if clinical improvement is not observed in 48 hours, the administration of loperamide should be discontinued. Patients with hepatic dysfunction should be monitored closely for signs of CNS toxicity because of the apparent high first - pass metabolism. The use of higher than the recommended doses for control of the diarrhoea may cause abnormal heart rhythms and serious cardiac events leading to death. However, in adult patients receiving the recommended dosage of loperamide, cases of syncope and ventricular tachycardia have been reported. Some of these patients were taking other drugs or had other risk factors that may have increased their risk of cardiac adverse reactions. Abuse and misuse of loperamide, as an opioid substitute, have been described in individuals with opioid addiction (see Overdose).

Use in children:

Loperamide is not recommended for children under 6 years of age. Its use has been associated with fatal episodes of paralytic ileus in infants and young children.

Usage in Pregnancy and Lactation:

Safe use of loperamide during pregnancy has not been established. It should only be used when clearly needed. It is not known whether loperamide is excreted in human milk. As a general rule, nursing should not be undertaken while a patient is on a drug since many drugs are excreted in human milk.

Contraindications:

It is contraindicated in patients with known hypersensitivity to the drug and in those with colon constipation. Loperamide HCl should not be used in patients with acute ulcerative colitis or pseudomembranous colitis associated with broad spectrum antibiotics. It should not be used as the primary therapy in acute dysentery, which is characterized by blood in stools and high fever.

Dosage:

Adults and Pediatrics 6 to 17 Years
Tablets
The tablet should be taken with liquid.
Acute diarrhoea: the initial dose is 2 tablet (4 mg) for adults and 1 tablet (2 mg) for children; followed by 1-2 tablet (2 mg) after every subsequent loose stool.
Chronic diarrhoea: the initial dose is 2 tablet (4 mg) daily for adults and 1 tablet (2 mg) daily for children; this initial dose should be adjusted until 1-2 solid stools a day are obtained, which is usually achieved with a maintenance dose of 1-6 tablets (2 mg-12 mg) daily.
The maximum dose for acute and chronic diarrhoea is 8 tablets (16 mg) daily for adults; in children it must be related to the body weight (3 tablets/20 kg) but should not exceed a maximum of 8 tablets per day.
Elderly
No dose adjustment is required for the elderly.
Renal Impairment
No dose adjustment is required for patients with renal impairment.
Hepatic Impairment
Although no pharmacokinetic data are available in patients with hepatic impairment, Loperamide HCl should be used with caution in such patients because of reduced first pass metabolism (see Warnings and Precautions).

Symptoms and Treatment for Overdosage and Antidote (s):

Overdosage of loperamide may result in constipation, CNS depression and gastrointestinal irritation. If vomiting occurs spontaneously upon ingestion, a slurry of 100g of activated charcoal should be administered orally as soon as fluids can be retained. A slurry of activated charcoal administered promptly after ingestion can reduce the amount of drug which is absorbed into the systemic circulation by as much as nine-fold.
If vomiting has not occurred, gastric lavage should be performed followed by administration of 100g of the activated charcoal slurry through the gastric tube. In the event of overdosage, patients should be monitored for signs of CNS depression for at least 24 hours. If CNS depression is observed, naloxone may be administered. If responsive to naloxone, vital signs must be monitored carefully for recurrence of symptoms of drug overdosage for at least 24 hours after the last dose of naloxone.
In view of the prolonged action of loperamide and short duration (1 to 3 hours) of naloxone, the patient must be monitored closely and treated repeatedly with naloxone as indicated.
In individuals who have intentionally ingested overdoses (reported in doses from 40 mg up to 792 mg per day) of loperamide HCl, prolongation of the QT/QTc interval, Torsades de Pointes, other ventricular arrhythmias and cardiac arrest, have been observed (see Warnings and Precautions). Fatal cases have also been reported. Abuse, misuse and/or overdose with excessively large doses of loperamide may inmask Brugada syndrome.

Storage Conditions: Store at or below 30°C.

Shelf-life: 2 years.

Pack size: A bottle of 50 and 80 tablets.

Blister pack: A box of 10 x 10, 50 x 10 and 100 x 10 tablets per strip.

Pack size (export only): A bottle of 500 and 1000 tablets.

Description: Round, convex, light blue, plain tablet with "SW" debossed on one side.

FURTHER INFORMATION CONCERNING THIS DRUG CAN BE OBTAINED FROM YOUR FAMILY PHYSICIAN / LOCAL GENERAL PRACTITIONER / PHARMACIST.

Manufacturer & Product Registration Holder:
SUNWARD PHARMACEUTICAL SDN. BHD.
No. 9, 11 & 17, Jalan Kempas 4,
Taman Perindustrian Tampoi Indah,
81200 Johor Bahru, Johor, Malaysia.

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